

Nursing care for the newborn and the mother after childbirth - Postpartum phase, home care

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Abstract- Childbirth is a joyous event when a woman gives birth to a child she desires. Despite the pain and embarrassment, childbirth is the long-awaited peak of pregnancy and the beginning of a new life. However, childbirth is also a critical time for the health of the mother and her baby. The postpartum phase is an essential period for a mother and her baby. The postpartum period includes a critical transitional time for a woman, her newborn, and her family, on a physiological, emotional, and social level.

The postpartum period, or puerperium, begins about an hour after birth and includes the following six weeks. Postpartum nursing care should respond to the needs of mother and baby during this particular phase which includes: prevention and early detection, treatment of complications and diseases, counseling, parental education and breastfeeding services, immunization, maternal nutrition and all other needs which lead to improved quality of maternal and infant health.

Problems such as postpartum (hemorrhage, pregnancy-related hypertension, pulmonary embolism and puerperal sepsis, decay of wounds, breast abscess and constipation, and sexual problems) may also be present during this period. Unknown postpartum disorders can lead to physical distress, psychological distress such as postpartum depression, and a poor quality of life for mothers who, if not treated promptly and effectively by nursing staff in collaboration with medical staff, may lead to deteriorating health. Postpartum care indicates the fact that most maternal deaths and disabilities occur during the postpartum period and that early neonatal mortality remains high, therefore home nursing care remains essential during this phase.

Key words: Postpartum phase, Nursing care, Home care, Complications, Parental education.

INTRODUCTION

Infant care activities can cause much anxiety for new parents. Support from nursing staff members can be an important factor in determining whether new parents seek and accept help in the future. Whether this child is the woman's or the couple's first newborn or an adolescent whose mother will be the primary caregiver, and whether or not the parents attended parenthood preparation classes, parents appreciate anticipatory guidance in the care of their infant. The nurse should avoid trying to cover all the content at one time because the parents can be overwhelmed by too much information and become anxious. Deficient knowledge should be identified before beginning to teach. Normal growth and development and the changing needs of the infant (for personal interaction and stimulation, growth milestones, exercise, injury prevention and social contacts), as well as the topics that follow, should be included during discharge planning with parents.

Complications in newborns in the postpartum phase:

Sleepy baby- During the first few days of life, some babies need to be awakened for feedings. Parents are instructed to be alert for behavior signs or feeding cues such as rapid eye movements under the eyelids, sucking movements, or hand- to-mouth motions. These signs, when present, indicate a good time to attempt breastfeeding. If the infant is awakened from a sound sleep, attempts at feeding are more likely to be unsuccessful. Unwrapping the baby, changing the diaper, sitting the baby upright, talking to the baby with variable pitch, gently massaging the baby's chest or back, and stroking the palms or soles may bring the baby to an alert state.

Fussy baby- Babies sometimes awaken from sleep crying frantically. Although they may be hungry, they cannot focus on feeding until they are calmed. Parents can swaddle the baby, hold the baby close, talk soothingly, and allow the baby to suck on a clean finger until calm enough to latch on to the breast. Placing the baby skin-to-skin with the mother can be very effective in calming the fussy infant. Infant fussiness during feeding may be the result of birth injury such as bruising of the head or fractured clavicle. Changing the feeding position may alleviate this problem. Persistent crying or refusing to breastfeed can indicate illness, and parents are instructed to notify the health care provider if either circumstance occurs. Ear infections, sore throat, or oral thrush may cause the infant to be fussy and not breastfeed well.

Slow weight gain - Newborn infants typically lose 5% to 10% of body weight before they begin to demonstrate weight gain. Weight loss of 7% in a breastfeeding infant during the first 3 days of life needs to be investigated. After the early milk has transitioned to mature milk, infants should gain approximately 110 to 200 g/week or 20 to 28 g/day for the first 3 months. Health care providers should evaluate and monitor infants who continue to lose weight after 5 days, who do not regain birth weight by 14 days, or whose weight is below the 10th percentile by 1 month. Parents are taught the warning signs of ineffective breastfeeding, including inadequate weight gain, minimal output, and feeding constantly. If any of these warning signs are present, the parent should notify the health care provider.

Jaundice- The type of jaundice most often seen in term newborns is physiologic jaundice. Hyperbilirubinemia is caused by bilirubin levels that rise steadily over the first 3 to 4 days, peak around day 5, and decrease thereafter. This condition has been called early-onset jaundice or breast-feeding-associated jaundice, which in the breastfed infant may be associated with insufficient feeding and infrequent stooling. Colostrum has a natural laxative effect and promotes early passage of meconium. Bilirubin is excreted from the body primarily through the intestines. To prevent early-onset, breastfeeding-associated jaundice, newborns should be breastfed frequently during the first several days of life. Increased frequency of feedings is associated with decreased bilirubin levels. To treat early-onset jaundice, breastfeeding is evaluated in terms of frequency and length of feedings, positioning, latch, and milk transfer. Factors such as a sleepy or lethargic infant or maternal breast engorgement may interfere with effective breastfeeding and should be corrected. Any breastfeeding infant who develops jaundice should be carefully evaluated for weight loss greater than 7%, decreased milk intake, infrequent stooling (less than three to four stools/day by day 4), decreased urine output (fewer than four to six wet diapers/day), and serum bilirubin levels.

Nursing care and education

Dressing and warmth -A room temperature of about 70°F or 21°C is warm enough for the infant. The infant should be dressed as the parents would like to be dressed, with a receiving blanket added. The abdomen should be checked to see if the infant is warm enough. The hands and feet may be slightly cooler than the rest of the body but should not be mottled or blue. The infant's head should be kept warm because many thermal skin sensors are located in the scalp. A hat is appropriate in the infant is outside when it is cold or windy.

Umbilical cord care- The cord is clamped immediately after birth. The goal of cord care is to prevent or decrease the risk of hemorrhage and infection. The umbilical cord stump is an excellent medium for bacterial growth and can easily become infected. The cord clamp is removed once the stump has started drying and is no longer bleeding, typically in 24 hours. Hospital protocol determines the technique for routine cord care. Common methods include the use of an anti- microbial agent such as bacitracin some experts advocate the use of alcohol alone, soap and water, sterile water, povidone-iodine, or no treatment (natural healing). A one-time application of triple dye has been shown to be superior to alcohol, povidone-iodine, or topical antibiotics in reducing colonization or infection; the use of alcohol is associated with prolonged cord drying and separation. The stump and base of the cord should be assessed for edema, redness, and purulent drainage with each diaper change. The nurse cleanses the cord and skin area around the base of the cord with the prescribed preparation (e.g., sterile water,

erythromycin solution, or triple-blue dye). The stump deteriorates through the process of dry gangrene; therefore odor alone is not a positive indicator of omphalitis (infection of the umbilical stump). Cord separation time is influenced by several factors, including type of cord care, type of birth, and other perinatal events. The average cord separation time is 10 to 14 days. Some dried blood may be seen in the umbilicus at separation. Doctors, midwives, or nurses add up these five factors for the Apgar score. Scores are between 10 and 0. Ten is the highest score possible, but few babies get it. That's because most babies' hands and feet remain blue until they have warmed up.

Apgar Scoring			
Appearance (skin color)	Normal color all over (hands and feet are pink) 2	Normal color (but hands and feet are bluish) 1	Bluish-gray or pale all over 0
Pulse (heart rate)	Normal (above 100 beats per minute) 2	Below 100 beats per minute 1	Absent (no pulse) 0
Grimace ("reflex irritability")	Pulls away, sneezes, coughs, or cries with stimulation 2	Facial movement only (grimace) with stimulation 1	Absent (no response to stimulation) 0
Activity (muscle tone)	Active, spontaneous movement 2	Arms and legs flexed with little movement 1	No movement, "floppy" tone 0
Respiration (breathing rate and effort)	Normal rate and effort, good cry 2	Slow or irregular breathing, weak cry 1	Absent (no breathing) 0

Positioning and Holding—The AAP Task Force on Infant Sleep Position and Sudden Infant Death Syndrome continues to recommend placing the infant in the supine position during the first few months of life to prevent sudden infant death syndrome (SIDS). Anatomically, the baby's flat, flat back allows the baby to rotate sideways in the prone position; therefore the lateral sleeping position is not recommended. When the infant is awake, "tummy time" can be provided under parental supervision so the infant may begin to develop appropriate muscle tone for eventual crawling; this tummy time is also effective in the prevention of a misshaped head (positional plagiocephaly).

Sponge Bathing and Skin Care—Bathing serves several purposes. It provides opportunities for (1) completely cleansing the infant, (2) observing the infant's condition, (3) promoting comfort, and (4) parent-child-family socializing. An important consideration in skin cleansing is the preservation of the skin's acid mantle, which is formed from the uppermost horny layer of the epidermis, sweat, superficial fat, metabolic products, and external substances such as amniotic fluid and microorganisms. At birth the skin has a pH of 6.4. Within 4 days the pH of the newborn's skin surface falls to within the bacteriostatic (pH less than 5). Consequently, only plain, warm water should be used for the bath during this 4-day period. Alkaline soaps (such as Ivory) and oils, powder, and lotions should not be used during this time because they alter the acid mantle, thus providing a medium for bacterial growth.

Rashes. Diaper rash—The majority of infants will develop a diaper rash at some time. This dermatitis or skin inflammation appears as redness, scaling, blisters, or papules. The warm, moist atmosphere in the diaper area provides an optimal environment for *Candida albicans* growth; dermatitis appears in the perianal area, inguinal folds, and lower abdomen. The usual source of infection is from handling by persons who do not practice adequate handwashing. It may also appear 2 to 3 days after an oral infection (thrush). Therapy consists of applications of an anticandidal ointment, such as clotrimazole or miconazole, with each diaper change.

Immunizations—The schedule for immunization should be reviewed with parents, HB vaccine is currently administered to newborns before hospital discharge or within 1 month of birth. Nurses should become familiar with this schedule and should provide written instructions to the parents about when and where to obtain immunization. An infant's ability to protect him or herself against antigens by the formation of antibodies develops sequentially, therefore the infant must be developmentally capable of responding to these antibodies, which is the reason for planning sequential immunizations for infants.

KALENDARI I OBLIGUAR I VAKSINIT, KOSOVË 2018/2019

MOSHA	LLOJI I VAKSINËS	MËNYRA E DHËNIES
Në lindje	BCG	I.D
	Hepatit B0	I.M
2 muaj	DPT1/HepB1/HiB1	I.M
	IPV1	I.M
3 muaj	DPT2/HepB2/HiB2	I.M
	IPV2	I.M
4 muaj	DPT3/HepB3/HiB3	I.M
	IPV3	I.M
12 muaj	DPTtr	I.M
	bOPVr	Orale
	MMR	S.C
6-7 vjeç	DT	I.M
	OPVr	Orale
	MMRr	S.C
12-13 vjeç	Td	I.M
	bOPVr	Orale
17-18vjeç	Td	I.M

Nursing care for the mother

The postpartum period, or puerperium, begins about an hour after birth and includes the following six weeks. Postpartum nursing care should respond to the needs of mother and baby during this particular phase which includes: prevention and early detection, treatment of complications and diseases, counseling, parental education and breastfeeding services, immunization, maternal nutrition and all other needs which lead to improved quality of maternal and infant health.

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Diet. In general, the breastfeeding mother should eat a healthy, well- balanced diet that includes an extra 200 to 500 calories per day over nonpregnant requirements. According to the IOM the estimated energy requirement for lactating woman during the first 6 months is 2700 kcal/day, during the next 6 months estimated energy requirement is 2768 kcal/day. In most cases the woman can consume a normal diet, according to her personal preferences and cultural practices. The ideal diet for lactating mother is well balanced, consisting of nutrient-dense foods. The intake of calcium, minerals, and fat soluble vitamins should be adequate. Woman may be told to continue taking their prenatal vitamins as long as they are breastfeeding. Mothers are encouraged to drink to quench thirst. Consumption of water by the mother does not increase milk supply, and overhydration can actually decrease milk production. **Weight loss-** Medications or diet that promote weight loss are not recommended for breastfeeding mothers. Many women will experience a gradual weight loss while lactating as fat stores deposited during pregnancy are used. This factor can be added incentive for breastfeeding. Another potential consequences of weight loss is reduced milk production. For most women, weight loss of 1 to 2 kilograms is safe, however, if weight loss exceeds this amount, careful evaluation of infant weight and feeding pattern is recommended.

Rest- The breastfeeding mother should rest as much as possible, especially in the first 1 or 2 weeks after birth. Fatigue, stress, and worry can negatively affect milk production and milk ejection (let-down). The nurse can encourage the mother to sleep when the baby sleeps. Breastfeeding in a side-lying position promotes rest for the mother. Assistance with household chores and caring for other children can be done by the father, grandparents or other relatives, and friends.

Breast care. The breastfeeding mother's normal routine bathing is all that is necessary to keep her breasts clean. Soap can have a drying effect on nipples; therefore the mother should avoid washing the nipples with soap. Breast creams should not be used routinely because they may block the natural oil secreted by the Montgomery glands on the areola. Modified lanolin with reduced allergens is safe to use on dry or sore nipples. Lanolin can be beneficial in moist wound healing of sore nipples. The breastfeeding mother may experience some common problems. In the majority of cases, these complications are preventable if the mother receives appropriate education about breastfeeding. Early recognition and prompt resolution of these problems is important to prevent interruption of breastfeeding and to promote the mother's comfort and sense of well-being. Emotional support provided by the nurse or lactation consultant is essential to help allay the mother's frustration and anxiety and to prevent care cessation of breastfeeding. Engorgement is response of the breasts to the sudden change in hormones and the onset of significantly increased milk volume. It usually occurs 3 to 5 days after birth when the milk "comes in" and lasts approximately 24 hours. The breasts are firm, tender, and hot and may appear shiny and taut. The areolae are firm, and the nipples may flatten, creating difficulty for the infant in latching on to the breast. Because back pressure on full milk glands inhibits milk production, if milk is not removed from the breasts, the milk supply may diminish. When engorgement occurs, it is a temporary condition that is usually resolved within 24 hours. The mother is instructed to feed every 2 hours, softening at least one breast, and pumping the other breast as needed to soften it. Pumping during engorgement will not cause a problematic increase in milk supply. Because of the swelling of breast tissue surrounding the milk ducts, ice packs are recommended in a 15 to 20 minutes on, 45 minutes off rotation between feedings. The ice packs should cover both breasts.

Teaching Feeding Techniques Position of the Mother and Infant. Both the mother and the infant must be positioned properly for optimal breastfeeding. Make the mother as comfortable as possible before she begins to nurse. Provide pain medications, if necessary. Pain or an awkward position may interfere with the let-down reflex and cause her to tire.

To increase her comfort, use pillows behind the mother's back, over an abdominal incision, and to support her arms. Her shoulders should be relaxed, and she should not be in a hunched position. Arrange folded blankets or pillows to elevate the infant to the level of the nipple and to prevent pulling and tension on the nipple, which would cause it to become sore. The infant's head and body should directly face the breast with the infant's nose, cheeks, and chin lightly touching the breast. If the infant must turn the head to reach the breast, swallowing is difficult. The neck should be flexed because hyperextension also makes swallowing difficult. The mother's hand position is also important. The palmar or C hand position is most often taught to new mothers. The mother holds her breast with her thumb on top and her fingers under the breast. Her little finger is against the chest wall and the other fingers provide support to the breast. Her fingers should be behind the areola and her thumb should not press on the breast enough to make the nipple tip upward, or the infant will suck improperly and the nipple may become sore. Some women use the "scissors hold" or "V" hold. The woman uses her index and middle fingers to support the breast.

She must be careful to place her fingers well behind the areola so her fingers do not slip down the wet areola and interfere with the placement of the infant's mouth. The mother should support her breast in place for the first few weeks.

Latch-On Techniques. Teach the mother techniques to help the infant latch on to the breast. The infant should be awake and hungry. Talking and cuddling can help a sleepy infant awaken and calm an upset infant. **Position of the Mouth.** Assess the position of the infant's mouth on the breast. The nipple and at least 2.5 to 3.8 cm (1 to 1.2 inches) of the areola should be in the infant's mouth to allow the nipple to be drawn toward the back of the mouth. This prevents the infant from sucking on the nipple only, which leads to sore nipples and insufficient milk production. **Suckling Pattern.** Teach the mother about the infant's suckling pattern. During nutritive suckling the infant sucks with smooth, continuous movements with only occasional pauses to rest. The infant may swallow after each suck or may suck several times before swallowing. Nonnutritive sucking often occurs when the infant is falling asleep. **Frequency of Feedings.** Breast milk moves through the stomach twice as fast as formula (Blackburn, 2007). Therefore infants are usually breastfed every 2 to 3 hours. Frequent feedings are especially important in the early days after birth, while lactation is being established and stomach capacity is small. Long periods between feedings increase the likelihood of breast engorgement. The resulting decreased stimulation of prolactin may reduce milk supply. Generally, the mother should nurse 8 to 12 times in each 24-hour period. Some infants vary the length of feedings and time between each feeding. **Length of Feedings.** Although early feedings were once limited to only a few minutes per breast to prevent sore nipples, improper positioning, rather than time at breast, is the usual cause of nipple trauma. When feedings are too short, infants receive little or no colostrum or milk. It may take as long as 5 minutes for the milk-ejection (let-down) reflex to occur during the early days after birth. Generally, mothers can allow infants to set the length of feedings. The infant should suckle vigorously for a period of time. **Feeding patterns with formula milk-** In the first 24 to 48 hours of life a newborn will typically take 10 to 15 ml of formula at a feeding. Intake gradually increases during the first week of life. Most newborns are drinking 90 to 150 ml at a feeding by the end of the second week, or sooner. The newborn infant should be fed at least every 3 to 4 hours, even if waking the newborn is required for the feedings; rigid feeding schedules, however, are not recommended. **Feeding technique-** Parents who choose formula feeding often need education regarding feeding techniques. Infants should be held for all feedings. During feedings, parents are encouraged to sit comfortably, holding the infant closely in a semi upright position with good head support. Feedings provide opportunities to bond with the baby through touching, talking, singing, or reading to the infant. Parents should consider feedings as a time of peaceful relaxation with the infant. A bottle should never be propped with a pillow or other inanimate object and left with the infant. This practice may result in choking, and it deprives the infant of important interaction during feeding. Teach parents to look for these cues and avoid overfeeding, which can contribute to obesity. Most infants swallow air when fed from a bottle and need a chance to burp several times during a feeding. Parents are taught various positions that can be used for burping. **Bottles and nipples.** Various brands and styles of bottles and nipples are available to parents. Most babies will feed well with any bottle and nipple. The bottles and nipples should be washed in warm soapy water, using a bottle and nipple brush to facilitate thorough cleansing. Boiling of bottles and nipples is not necessary unless some question exists about the safety of the water supply or if the infant has oral thrush.

Complications in mothers after childbirth can be numerous and requires close cooperation with the doctor in the rehabilitation of the woman. Complications that may occur are: Postpartum Hemorrhage (PPH), Hypertensive Disorders in Pregnancy (HDP), Obstetric Pulmonary Embolism, Puerperal Sepsis, Heart Disease, Depression, Anemia, Fatigue, Headache, Back Pain, Perineal Trauma, Urinary problems, Urinary Tract Infection (UTI), Wound rupture (in cesarean births), Constipation, Hemorrhoids. All these problems need proper consultation from the specialist and we as nurses should do nursing care by administering the therapy given by the specialist improving the woman's health and quality of life.

Also other problems can be breast problems, where many problems appear and they are: Sore nipples, Monilial infections, Plugged milk ducts, Mastitis. So in cooperation with the doctor we can treat these problems by doing the right treatment in the administration of therapy and breast care so that the mother breastfeeds the baby. For sore nipples and plugged milk ducts the nurse should advise the mother to use fresh leaves and raw cabbage placed on the breasts can help reduce swelling. Cabbage leaves are washed, cooled in the refrigerator, and then placed on the breasts for 15 to 20 minutes. This treatment can be repeated for two or three sessions. Then washing with hot water helps a lot in freeing the milk ducts, also breast massage gives a very good effect. For sore nipples, it is advisable for mothers not to use different creams but to keep them clean by often washing them with water and massaging them with their milk.

POSTPARTUM PSYCHOLOGIE COMPLICATIONS. Mental health disorders have implications for the mother the newborn, and the entire family. Such conditions interfere family integration and some may threaten the safety and well-being the mother, newborn, and other children. Mood Disorders. Mood disorders are the predominant mental health disorder in the postpartum period, typically occurring within 4 weeks of childbirth. Many women experience a mild depression, or "baby blues," after the birth of a child. Others can have more serious depressions that can eventually incapacitate them to the point of being unable to care for themselves or their babies. Nurses are strategically positioned to offer anticipatory guidance, to assess the mental health of new mothers, to offer therapeutic interventions, and to referring when necessary. Failure to do so may result in tragic consequences. The Diagnostic and Statistical Manual of Mental Disorders contains the official

guidelines for the assessment and diagnosis of psychiatric illness. However, specific criteria for postpartum depression (PPD) are not listed. Instead, postpartum onset can be specified for any mood disorder either without psychotic features (i.e., PPD) or with psychotic features (i.e., postpartum psychosis) if the onset occurs within 4 weeks of childbirth (APA). **Postpartum depression with psychotic features-** Postpartum psychosis is a syndrome most often characterized by depression (as described previously), delusions, and thoughts by the mother of harming either the infant or herself. A postpartum mood disorder with psychotic features occurs in 1 to 2 per 1000 births and may occur more often in primiparas. Once a woman has had one postpartum episode with psychotic features, a 30% to 50% likelihood of recurrence exists with each subsequent birth. Symptoms often begin within days after the birth, although the mean time to onset is 2 to 3 weeks and almost always within 8 weeks of birth. Characteristically, the woman begins to complain of fatigue, insomnia, and restlessness and may have episodes of tearfulness and emotional lability. Complaints regarding the inability to move, stand, or work are also common. Later, suspiciousness, confusion, incoherence, irrational statements, and obsessive concerns about the baby's health and welfare may be present. Delusions may be present in 50% of all women with this disorder and hallucinations in approximately 25%. Auditory hallucinations that command the mother to kill the infant can also occur in severe cases. When delusions are present, they are often related to the infant. The mother may think the infant is possessed by the devil, has special powers, or is destined for terrible fate. Grossly disorganized behavior be exhibited as a disinterest in the infant or an inability to provide care. Some women will insist that something is Wrong with the baby or accuse nurses or family members of hurting or poisoning him or her. A specific illness included in depression with psychotic features is bipolar disorder. **Medical management.** A favorable outcome is associated with a good premorbid adjustment (before the onset of the disorder) and a supportive family network. Because mood disorders are usually episodic, women may experience another episode of symptoms within a year or two of the birth. Postpartum psychosis is a psychiatric emergency, and the mother will probably need psychiatric hospitalization. Antipsychotics and mood stabilizers such as lithium are the treatments of choice. If the mother is breastfeeding, some sources advise caution while prescribing some agents. Antipsychotics and lithium should be avoided in breastfeeding mothers, but other mood stabilizers may be compatible with breastfeeding. Having contact with her baby is usually advantageous for the mother if she so desires, but visits must be closely supervised. Psychotherapy is indicated after the period of acute psychosis is past. **CARE MANAGEMENT-** Even though the prevalence of PPD is fairly well established, it often remains undetected because women are hesitant to report symptoms of depression even to their own health care providers. **Nursing Interventions-** Nursing care in the postpartum period assesses for any signs of boredom and makes fun assessments as needed. PPD must be discussed by nurse to prepare new parents for potential problems in the postpartum period. The family must be able to recognize the symptoms and know where to go for help. Written materials that explain what the woman can do to prevent depression could be used as part of discharge planning. Mothers are often discharged from the hospital before the blues or depression occurs. If the postpartum nurse is concerned about the mother, a mental health consultation should be requested before the mother leaves the hospital. Routine instructions regarding PPD should be given to the person who comes to take the woman home; for example, "If you notice that your wife (or daughter) is upset or crying a lot, please call the postpartum care provider immediately- don't wait for the routine postpartum appointment. **NURSING ALERT -** Because the newborn may be scheduled for a checkup before the mother's 6-week checkup, nurses in well-baby clinics or pediatrician offices should be alert for signs of PPD in new mothers and be knowledgeable about community referral rescue. **PATIENT INSTRUCTIONS FOR SELF-MANAGEMENT-** Activities to Prevent Postpartum Depression • Share knowledge about postpartum emotional problems with close family and friends. • Take care of yourself: Eat a balanced diet, exercise on a regular basis, and get enough sleep. Ask someone to take care of the baby so that you can get a full night's sleep. • Share your feelings with someone close to you; do not isolate yourself at home. • Do not overcommit yourself or feel as though you need to be a superwoman. • Do not place unrealistic expectations on yourself. • Do not be ashamed of having emotional problems after your baby is born, it happens to approximately 15% of women.

The room of the mother and the newborn

It is recommended that the room be clean and well ventilated, have a normal temperature of 21°C, be well lit by sunlight, because they are very important for the baby, especially in preventing the appearance of physiological jaundice.

The sheets and covers should be well washed and without the placement of any scented shampoo as it may cause allergies in the baby. Room visits should be avoided as much as possible as the baby is very sensitive and at risk of getting an infection, considering these days the Covid 19 virus pandemic is also prevalent, so visits should be reduced. It is also preferable that the room does not contain fragrant odors and also that there are no flowers placed in vases as it may cause allergies in the baby and mother.

All this information should be given by the nurse in order to prevent various infections that may be inside the room where the mother and baby are staying. Our task as health care educators is to prevent, treat and care, us properly as nurses and health educators at the same time.

PURPOSE OF THE STUDY

- Through this paper to create a program for care for children and mothers after childbirth, because one is missing in Kosovo.
- How should we as nurses counsel, care and educate young mothers in coping with the postpartum phase.
- With nursing care to create good health well-being, good quality of life and help in coping with every complication that the mother and the newborn have during the postpartum phase.

HYPOTHESIS

- Many mothers in the postpartum stage have many problems and complications in which they have difficulty going through them alone.
- Nursing care in collaboration with other medical staff are the factors that would affect the improvement of the quality of health of mother and child during the postpartum phase.

LIMITATIONS

Our paper had a limitation, and that is the failure to do statistical research, where we would have surveyed mothers after discharge from the hospital for all their needs in the postpartum phase, due to the outbreak of the Covid 19 virus pandemic, and we have not been able to do statistical research. But our paper is based on the current situation in Kosovo and seeing the need for new mothers who need special care, we have come to the conclusion that through this paper to take more seriously the postpartum phase from local institutions and start having a program where all mothers would be supported during this rather complicated period.

DISCUSSION AND CONCLUSION

The puerperium is an important period for post delivery mothers. Many complications following delivery can occur during this period up to six weeks postpartum. Unrecognized postpartum disorders can lead to physical discomfort, psychological distress, low self esteem and poor quality of life for the mothers. It is the duty of health care workers to provide quality postnatal care and to be able to identify the problems earlier so that proper intervention can be initiated. With good health care support, hopefully most mothers will be able to achieve full recovery to the pre pregnancy state sooner. A special phase involves special needs. Only scanty research data are available on the needs of women and babies in the postpartum period. Partly based on the data in the literature, results the needs of women and infants as follows:

In the postpartum period women need:

- ☐ information/counselling on care of the baby and breastfeeding, what happens with and in their bodies - including signs of possible problems, self care - hygiene and healing, sexual life, contraception, nutrition
- ☐ support from health care providers, partner and family – in the emotional and psychological shape, health care for suspected or manifest complications, time to care for the baby, help with domestic tasks, maternity leave, social reintegration into her family and community, protection from abuse/violence.

Women may fear: inadequacy, loss of marital intimacy, isolation, constant responsibility for care for the baby and others.

Newborn infants need: easy access to the mother, appropriate feeding, adequate environmental temperature, a safe environment, parental care, cleanliness, observation of body signs by somebody who cares and can take action if necessary, access to health care for suspected or manifest complications, nurturing, cuddling, stimulation, protection from disease, harmful practices, abuse/violence, acceptance of sex, appearance, size, recognition by the state (vital registration system).

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